

## CLINICAL NOTE – FICTITIOUS PATIENT DATA (FOR TESTING ONLY)

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Patient: Wei-Lin Huang  
Date of Birth: 03 September 1948  
MRN: 4481920  
SSN: 512-63-9047  
Address: 88 Jade Garden Court, San Francisco, CA 94108  
Phone: (415) 667-3921  
Email: weilin.huang48@gmail.com  
Referring Physician: Dr. Mei-Xing Loh, NPI 6612039485  
Facility: UCSF Medical Center, San Francisco

Visit Date: 14 June 2026  
Reason for Visit: Palpitations and progressive exertional dyspnoea – known atrial fibrillation, query decompensated heart failure.

### HISTORY OF PRESENT ILLNESS

Wei-Lin Huang is a 77-year-old male, originally from Guangzhou, China, with a background of permanent atrial fibrillation and ischaemic cardiomyopathy presenting with a 10-day history of worsening dyspnoea on exertion (now limiting to < 50 metres), bilateral ankle oedema, and two episodes of nocturnal orthopnoea. He denies chest pain or syncope. His son Kevin Huang (contact: 415-883-2047) is his primary carer. His wife Mei-Fang Huang (home: 415-667-3921) has limited English – Kevin acts as interpreter. GP referral from Dr. Mei-Xing Loh (clinic: 415-220-8844).

### PAST MEDICAL HISTORY

- Permanent atrial fibrillation (diagnosed 2014, on anticoagulation)
- Ischaemic cardiomyopathy (EF 35%, last echo 2024)
- Hypertension (diagnosed 2003)
- Type 2 diabetes mellitus (HbA1c 7.9% at last review, February 2026)
- CKD Stage 3a (eGFR 52 mL/min/1.73m<sup>2</sup>)
- Prior NSTEMI (2019, medically managed – no PCI)
- OSA (on CPAP since 2021)

### CURRENT MEDICATIONS

- Apixaban 5mg twice daily (anticoagulation for AF)
- Bisoprolol 5mg once daily (rate control)
- Furosemide 40mg once daily (diuretic)
- Sacubitril/Valsartan 97/103mg twice daily (HFrEF)
- Spironolactone 25mg once daily
- Empagliflozin 10mg once daily
- Atorvastatin 40mg once daily
- Metformin 500mg twice daily

### ALLERGIES

Ramipril (dry cough – intolerable), Sulfonamides (rash)

### VITAL SIGNS

BP: 158/96 mmHg HR: 88 bpm (irregularly irregular) RR: 22 breaths/min  
SpO2: 91% on room air -> 95% on 2L nasal cannula Temperature: 36.8 degrees C  
Weight: 74 kg (up 6 kg from baseline 68 kg – 10 days) BMI: 26.2 kg/m2  
JVP: Elevated 5 cm above sternal angle

## LABS

Hb: 118 g/L (mild normocytic anaemia)  
WBC:  $9.4 \times 10^9/L$  Platelets:  $214 \times 10^9/L$   
BNP: 1,842 pg/mL (markedly elevated – consistent with decompensation)  
Troponin I: 0.08 ug/L (mildly elevated – chronic elevation pattern)  
Creatinine: 148 umol/L (elevated, up from baseline 112 umol/L)  
eGFR: 38 mL/min/1.73m2 (acutely worsened – cardiorenal syndrome)  
Na: 133 mmol/L K: 4.9 mmol/L Albumin: 31 g/L HbA1c: 8.3%  
LFTs: ALT 68 U/L, AST 74 U/L (mildly elevated – hepatic congestion)

## IMAGING

CXR: Cardiomegaly. Bilateral perihilar haziness. Upper lobe venous diversion.  
Bilateral small pleural effusions. Kerley B lines at right base.  
Findings consistent with acute pulmonary oedema.

Echo (today): EF 28% (worsened from 35% in 2024). Severe LV dilation.  
Global hypokinesis. Grade III diastolic dysfunction. Moderate MR.  
Estimated RVSP 48 mmHg. Small pericardial effusion.

## ASSESSMENT

Decompensated heart failure (HFrEF, EF 28%) – acute-on-chronic exacerbation.  
Atrial fibrillation – permanent, rate controlled.  
Cardiorenal syndrome type 1 – acute AKI in context of cardiac decompensation.  
Dilutional hyponatraemia – secondary to fluid overload.

## PLAN

1. Admit to Cardiology ward – HDU monitoring (telemetry, strict fluid balance).
2. IV Furosemide 80mg bolus then 10mg/hr infusion – target 1-1.5L negative daily.
3. Continue Sacubitril/Valsartan – do NOT uptitrate until euvolaemic.
4. Continue Apixaban – no dose change (renal dosing already applied).
5. Hold Empagliflozin during acute admission (risk of euglycaemic DKA).
6. Hold Metformin – creatinine elevated, eGFR < 45.
7. Repeat echo in 48 hours post-diuresis to reassess EF and effusions.
8. Discharge planning: Kevin Huang to be counselled on daily weight monitoring.
9. Home nursing follow-up at 88 Jade Garden Court.
10. Patient ID wristband: 4481920.

Signed: Dr. Rachel Okonkwo, Registrar Cardiology

Date: 14 June 2026

Facility Contact: UCSF Medical Center, 505 Parnassus Ave, San Francisco CA 94143